

Scheduler's Recovery — the Nurse Calls Out

Wednesday morning. The SN from yesterday's plan calls out sick. Her five visits — SOC, OASIS discharge, three routine — are now stranded, and the scheduler has to recover the day before the first door.

STRANDED CASELOAD — 5 VISITS · 7.25 PTS

SOC · Mrs. Alvarez
2.5 · RN · West Ashley

OASIS D/C · Mr. Bell
1.75 · RN · Mt Pleasant

Routine · Mrs. Chen
1.0 · wound · West Ashley

Routine · Mrs. Evans
1.0 · CHF/dementia · Johns Is.

Routine · Mr. Davis
1.0 · MRSA · Downtown

01
Log the call-out

Open the coverage workflow; confirm scope — out today, and is tomorrow at risk too?

→

02
Triage by hard deadline

Which visits can't slip (SOC window, discharge order) vs. reschedulable inside their frequency window.

→

03
Read today's open capacity

Other RNs, per-diem / float; assessing-RN capacity specifically for the OASIS visits.

→

04
Cover the non-negotiable, slide the rest

Match discipline · competency · geography · continuity for anything that must happen today.

→

05
Notify & re-confirm

Patients, caregiver-present windows, and the care team — before anyone drives.

→

06
Rebalance & document

Protect covering clinicians' ceilings; check Thursday's load; missed-visit notes only if truly uncovered.

1

COVER TODAY

SOC · Mrs. Alvarez
29407 · West Ashley · 2.5 pts

Assign per-diem RN (Nia P.) — AM opening, West Ashley territory. Re-confirm the 8–11a caregiver window with the new nurse and call the daughter now.

REGULATION

SOC 48-hr window expires TODAY — cannot slip; OASIS comprehensive + F2F required

COVERAGE

Assessing-RN only — pull from per-diem / float; confirm assessing capacity today

PATIENT / CAREGIVER

Caregiver home 8–11a → new nurse must hit that window; call the daughter

GEOGRAPHY

West Ashley — match a near-territory RN to protect the morning slot

COST

Per-diem = premium labor; justified by the hard SOC deadline

CONTINGENCY — IF NO RN CAN TAKE THE SOC IN THE CAREGIVER WINDOW

Escalate to the DCS: **(1)** the DCS (an RN) performs the SOC herself · **(2)** borrow an RN from a sister branch · **(3)** last resort — the SOC slips: document, notify the referral source, and flag the at-risk admission. This is the one visit that must not be dropped.

2

RESCHEDULE THU

OASIS Discharge · Mr. Bell
29464 · Mt Pleasant · 1.75 pts

Slide to Thursday AM — within the OASIS-Discharge window (≤ 2 days of the discharge date). Hold for the nurse's return; notify Mr. Bell.

REGULATION

OASIS D/C allowed within 2 days of discharge → Thursday is in-window

COVERAGE

RN required — keep for the nurse to avoid a substitute doing the discharge OASIS

GEOGRAPHY

Mt Pleasant (far east) — batch with another east-side visit Thursday

3

RESCHEDULE THU

Routine · Mrs. Chen — wound
29414 · West Ashley · 1.0 pt

Reschedule to Thursday AM (her preference) — 3×/wk spacing still met, no frequency gap. Confirm dressing supplies on hand.

REGULATION

Ordered 3×/wk — spacing holds if seen Thursday; no gap exception

COVERAGE

Would need wound competency to cover today — not worth a substitute for one day

PATIENT / CAREGIVER

Prefers mornings — offer Thu AM

GEOGRAPHY

West Ashley

4

RESCHEDULE THU

Routine · Mrs. Evans — CHF (dementia)
29455 · Johns Island · 1.0 pt

Reschedule to Thursday with the same nurse — continuity matters for dementia. Confirm the husband is present; call ahead to reassure (anxious).

CONTINUITY

Dementia → a substitute is disruptive; keep the same nurse → reschedule, don't cover

REGULATION

Ordered 1×/wk — inside the window through Saturday

PATIENT / CAREGIVER

Husband must be present; anxious patient — proactive call

COVERAGE

If a CHF check is acuity-flagged, send a covering RN today instead

5

RESCHEDULE THU

Routine · Mr. Davis — med mgmt (MRSA)
29403 · Downtown · 1.0 pt

Reschedule to Thursday (a non-dialysis day). Sequence last again; PPE.

PATIENT / CAREGIVER

Dialysis Mon/Wed/Fri 1–4p → he's unavailable this afternoon anyway

REGULATION

Ordered within window; contact isolation → sequence last

GEOGRAPHY

Downtown

COST

Lowest urgency — safe to slide

THEN THE SCHEDULER ALSO...

Confirms whether the nurse is out **tomorrow too** — if so, Thursday's four slid visits need real coverage, not just her return. Re-checks **Thursday's load** (her existing caseload + 4 slid visits can blow the ~7-pt ceiling → distribute or offload to per-diem). Updates the roster / availability and logs the sick day. **No missed-visit documentation** — everything is covered or rescheduled in-window; notes are only filed if a visit truly goes undelivered.

Outcome: SOC covered today (per-diem RN) · D/C + 3 routines rescheduled to Thursday in-window · 5 patients & caregivers notified · Thursday load rebalanced · zero missed-visit exceptions.

Illustrative recovery built from the Capacity & Scheduling variable model (coordination & coverage layer). Fictional patients; the disposition logic is the real path a scheduler runs on a same-day call-out.